جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عبير فكرى حسين محمد
المحافظة : القليوبيه **الحي / المدينة :** شبرا الخيمه اول **العنوان :** ..
رقم البطاقة : 27907011409188 **رقم التليفون :** 01274947066 **تاريخ تسجيل الإستمارة :** 14-10-2026
العمر(تاريخ الميلاد) : 01-07-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
الإخصائي الاجتماعي
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج كيماوي تاكسان (دوسيتاكل) [14152] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عبير فكرى حسين محمد
National ID: 27907011409188

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Docetaxel chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٧٩٠٧٠١١٤٠٩١٨٨ ٢٠٢٠/١٠
ربة منزل

أنثى مسلمة متزوجة
احمد السعيد محمد السيد
١١ البطاقة سارية حتى ٢٠/١٠/٢٠٢٧

٠١٢٧٤٩٤٧٠٦٦
٠١١٠٢٤٦٤١٣٧

جمهورية مصر العربية
بطاقة تحقيق الشخصية



عبير
فكرى حسين محمد
٢٣ش صلاح الدين الا يوبى شبرا البلد
شبرا الخيمه اول - القليوبية

٢ ٧٩ ٠٧ ٠١ ١٤ ٠٩١ ٨٨

IP3051660

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	**عبير فكرى حسون محمد 0**	**Code Number**	**YK 262999**
Sex/Age	Female 46Y	**Patient ID**	**20118**
Registered Date	**24/05/2026 09:28:18**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	01/06/2026 21:09:32	**Branch**	**Gustave Roussy**
Reporting Date	**6/11/2026, 11:58:25 AM**		

Pathology Report

Pathology Report

Clinical Data: Left breast mass, BIRADs 5.

Nature of Specimen: Tru-cut biopsy, collected on 24/05/2026, at 1:40 pm, Cold ischemia time: 20 seconds.

Gross Examination: Six grey white tissue cores, ranging in length 1-1.5 cm, totally submitted.

Microscopic Examination:

* Histological type: Invasive duct carcinoma.
* Grade: 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
* Ductal carcinoma in situ: Absent.
* Lobular carcinoma in situ: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.
* Tumor infiltrating lymphocytes (TILs): Mild

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2. (B5b)
- For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Maha Guimei

Liver Function

:

{

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: عبير فكرى حسين محمد	
MR. : 11301	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is 47 years old with regular menstrual cycles. There is no reported family history of breast cancer and the patient has a history of hypertension.

Core needle biopsy demonstrated invasive ductal carcinoma, grade 1 1. Immunohistochemistry showed estrogen receptor 0/8, progesterone receptor 0/8, HER2 score 3, and Ki-67 of 20 %. Contrast-enhanced spectral mammography revealed diffuse skin thickening in the left breast extending from the 11 to 3 o'clock position with intraductal extension. Staging imaging, including computed tomography and bone scan, showed no evidence of distant metastasis.

The referring team recommended consideration of neoadjuvant chemotherapy given a clinical stage of T4b N1, followed by left mastectomy. The multidisciplinary team agreed to proceed with placement of a port-a-cath, performance of echocardiography and laboratory studies, and administration of neoadjuvant chemotherapy with the TCHP regimen for six cycles, after which a left total mastectomy will be performed.

تحريرا في ,

التاريخ :	23-09-2026	الوقت :	

الطبيب المعالج :

يعتمد ,

جمهورية مصر العربية
بطاقة تحقيق الشخصية



عبير
فكرى حسين محمد
٢٢ش صلاح الدين الا يوبى شبرا البلد
شبرا الخيمه اول - القليوبية

٢٧٩٠٧٠١١٤٠٩١٨٨

IP3051660

٢٧٩٠٧٠١١٤٠٩١٨٨ ٢٠٢٠/١٠
ربة منزل

انثى مسلمة متزوجة
احمد السعيد محمد السيد
١١ البطاقة سارية حتى ٢٠٢٧/١٠/٢٠

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient's Name:	**Abeer Fikri Hussein Mohammed**
Date:	**2026-05-21**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

جوستاف روسي

BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH :RESOLUTION BREAST SONOGRAPHY REVEALED

CLINICAL SUMMARY:
Left breast palpable lump

FINDINGS:

LEFT BREAST:
- Dense parenchyma, (ACR C).
- The upper midline is the seat of an ill-defined spiculated mass associated with pleomorphic calcifications within, complimentary U/S demonstrated three underlying interconnected heterogenous hypoechoic solid masses located at 12-2 o'clock, seen reaching the nipple and laying about 7.5mm, collectively measuring about 7x5cm.
- Mottle parenchyma is also noted.
- Benign calcifications are also noted.
- Few level I pathological axillary nodes are seen, reaching 2.8x1.3cm.

RIGHT BREAST:
- Dense parenchyma, (ACR C).
- No mass, spiculation or suspicious micro calcifications, this was further confirmed by an U/S examination which depicted mottled parenchyma and few small scatted cysts.
- Benign calcifications are also noted.
- Normal skin/nipple/areola complex.
- Non specific axillary nodes are noted.

CONCLUSION:
Left breast (BIRADS 5): UOQ malignant process as described, (for biopsy), and adenosis
Right breast (BIRADS 2): Adenosis

Best regards,
Dr. Mahi Mokhtar

Mammogram
[[DOCTYPE:MAMMOGRAM]]

جوستاف روسي

إسم المريض ثلاثي: .....
رقم الملف: .....

تقريـــر طبـــي

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشـان تقرير طبـي للمـريض: ...مسز عكرى.....

بقسم.....رقم طبي .....

نتشرف بأن نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول..... .....تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

Breast Ca
Her-2+3
CT CAP free & Bone Scan
free
for Phesgo

تحريرا في

الوقت: ..... **التاريخ :** / /

الطبيب المعالج

.....

يعتمد ،،

جمهورية مصر العربية
وزارة الصحة و السكان
المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة

خطاب إداري

الرقم القومي	٢٧٩٠٧٠١١٤٠٩١٨٨
إسم المواطن	عبير فكرى حسين محمد

تاريخ اللجنة ٢٠٢٦-٠٩-٢١	رقم الطلب	٦٧٩٢٣١٠٦
	الحالة	خطاب إداري
	نص الخطاب	
	نوافي بالابحاث الخاصه بالمريض الداله علي التشخيص حديثة موضحا بها بيانات المريض والتاريخ .	
جهة الإرسال جوستاف روسي انترناشيونال - مصر (EPE) *	٢٨٦٦٤ / ٥٩٠٩ / ٢١٨٢٢	

تحريراً في ٢١-٠٩-٢٠٢٦

٢٨٦٦٤ / ٥٩٠٩ / ٢١٨٢٢

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

Patient ID: 97030	Patient Name English: Abeer Fekry Hussein Mohamed
٤٧ سنة Age:	2026/07/08 11:19:26 AM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A- 46- years old female patient with pathologically proven invasive ductal carcinoma grade II (-ve ER, -ve PR, +ve HER2 and Ki67= 20 %)

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild nodular background enhancement.
ACR: C (Heterogeneously dense breast parenchyma)

LEFT breast:

- **Low energy (Non contrast) images show**: Mid upper irregular shaped spiculated dense lesion associated with architectural distortion, underlying microcalcifications and diffuse skin thickening.
- **Recombined (Post contrast) images show**:
  - Corresponding large 11-12 o'clock irregular spiculated heterogeneously enhancing mass lesion with marked conspicuity, measuring about 4.6 x 6.5 cm along the maximum dimensions, associated with linear non-mass enhancement denoting intraductal extension reaching the nipple.
  - Multiple smaller heterogeneously enhancing masses of moderate conspicuity are scattered through the UOQ, retroareolar region and surrounding the index lesion along with intervening non-mass enhancement. The overall area of the enhancement (including the main index lesion and the surrounding lesions) measures about 12 cm along its maximum dimension (extending from 11:00-3:00).

RIGHT breast:

- **Low energy (Non contrast) images** No asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images**: no pathological mass or non-mass enhancement.
- Normal skin / nipple / areolar complex.

IMPRESSION:

- **LEFT BREAST:** Upper outer quadrant and retroareolar (from 11:00-3:00) multiple pathologically enhancing mass lesions associated with intraductal extension reaching the nipple, and diffuse edematous changes, as above described (BIRDAS VI)
- **RIGHT BREAST**: No pathological mass or non-mass enhancement (BIRADS I).

Best regards,

Dr. Sherihan fakhry , MD
Dr. Hekmat Samir, MD

Patient's Name:	**Abeer Fikri Hussein**
Date:	**2026-06-22**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- **New case of left breast cancer for metastatic work up.**

Findings:

- Left breast central mass is seen measuring 4x3.5cm.
- Increased skin thickness of left breast.
- Left axillary enlarged LNs largest 20mm.
- Clear right breast and right axilla.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Left thyroid lobe small nodule measuring 1cm.

OPINION:

- Left breast central mass.
- Increased skin thickness of left breast.
- Left axillary enlarged LNs.
- Clear right breast and right axilla.
- No lung metastasis.
- Left thyroid lobe small nodule.

Patient's Name:	**Abeer Fikri Hussein**
Date:	**2026-06-22**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of enlarged size 20cm with fatty homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- D4 and D12 vertebral body lytic lesions.
- Right acetabular sclerotic lesion.

Opinion:

- **Fatty hepatomegaly with no hepatic metastasis.**
- **D4 and D12 vertebral body lytic lesions.**
- **Right acetabular sclerotic lesion.**
- **For bone scan.**

Dr. Mai Shaaban MD

Dr.Mai Shaaban

جوستاف روشي

8/18

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

08/18/2026

MDT FORM

Name:	خير نكرى حسين محمد	MRN:	11301
Age:	47 Y	ID:	27907011409189
Menopausal Sta	regular	Phone:*	01272896036
NRS SCORE:		Marital Status:	married
BMI:		Family History:	-VE
Gender			
CO Morbidity:	htn	Reason Of MDT:	NEW
Surgical History:*	nad	ECOG Performance Status	○ 0◉ 1○ 2○ 3○ 4

Clinical Data:

BX: idc g11

ER: 0/8 PR: 0/8 HER2: 3 KI67: 20

History:

Sonomammography:
cesm :lt diffuse skin thickening area 11-3 oclock with intraductal extension

Biomarker/Molecular

R L



TNM

	Right Side				Left Side			
	Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Mets Workup:
ct :free
bone scan free

Recommendations:
in view of being T4B N1 FOR possibility of NACT then lt MRM

Clinical Staging:

Pathological Staging:

MDT Outcome & Plan
for
porta Cath insertion
ECHO , Labs
NACT TCHP x6 then >> LT MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د هنان حزت / . گوانتی	د محمد نفروی		

GUSTAVE ROUSSY

Patient Name	عبير فكرى حسين محمد	Case No.	262999
Sex/Age	Female 46 Y	Patient No.	20118
Registered Date	24/5/2026 09:28:18 AM	Referred By Dr.	Interventional Radiology
Validation Date	25/6/2026 04:00:20 PM	Branch	Gustave Roussy
Reporting Date	28/6/2026 03:22:12 PM	Pathology No.	1657/26

Pathology Report

Clinical Data: Left breast mass, BIRADs 5.

Nature of Specimen: Tru-cut biopsy, collected on 24/05/2026, at 1:40 pm, Cold ischemia time: 20 seconds.

Gross Examination: Six grey white tissue cores, ranging in length 1-1.5 cm, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade: 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
* Ductal carcinoma in situ: Absent.
* Lobular carcinoma in situ: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.
* Tumor infiltrating lymphocytes (TILs): Mild

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 20%.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2. (B5b)

IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 20%.

GUSTAVE ROUSSY INTERNATIONAL

Gustave Roussy International Egypt Pathology Lab
جوستاف روسي انترناشونال - مصر معمل الباثولوجي

جوستاف روسي

Patient Name	عبير فكرى حسين محمد	Case No.	262999
Sex/Age	Female 46 Y	Patient No.	20118
Registered Date	24/5/2026 09:28:18 AM	Referred By Dr.	Interventional Radiology
Validation Date	25/6/2026 04:00:20 PM	Branch	Gustave Roussy
Reporting Date	28/6/2026 03:22:12 PM	Pathology No.	1657/26

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

** *Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Maha Guimei.
Dr. Walaa Ibrahim Ghonaim.

Patient ID: 97030	Patient Name English: Abeer Fekry Hussein Mohamed
Age: ٤٦ سنة	2026/07/08 11:19:26 AM :Report Date

Contrast enhanced Spectral Mammography

INICAL SUMMARY: A- 46- years old female patient with pathologically proven invasive ductal :inoma grade II (-ve ER, -ve PR, +ve HER2 and Ki67= 20 %)

CHNIQUE: CC & MLO low energy & recombined (post contrast) images

DINGS:

Background Uptake: mild nodular background enhancement.

ACR: C (Heterogeneously dense breast parenchyma)

FT breast:

- **Low energy (Non contrast) images show:** Mid upper irregular shaped spiculated dense lesion associated with architectural distortion, underlying microcalcifications and diffuse skin thickening.
- **Recombined (Post contrast) images show:**
  - Corresponding large 11-12 o'clock irregular spiculated heterogeneously enhancing mass lesion with marked conspicuity, measuring about 4.6 x 6.5 cm along the maximum dimensions, associated with linear non-mass enhancement denoting intraductal extension reaching the nipple.
  - Multiple smaller heterogeneously enhancing masses of moderate conspicuity are scattered through the UOQ, retroareolar region and surrounding the index lesion along with intervening non-mass enhancement. The overall area of the enhancement (including the main index lesion and the surrounding lesions) measures about 12 cm along its maximum dimension (extending from 11:00-3:00).

GHT breast:

- **Low energy (Non contrast) images** No asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images:** no pathological mass or non-mass enhancement.
- Normal skin / nipple / areolar complex.

PRESSION:

- **LEFT BREAST:** Upper outer quadrant and retroareolar (from 11:00-3:00) multiple pathologically enhancing mass lesions associated with intraductal extension reaching the nipple, and diffuse edematous changes, as above described (BIRDAS VI)
- **RIGHT BREAST:** No pathological mass or non-mass enhancement (BIRADS I).

Best regards,

Dr. Sherihan fakhry , MD

Dr. Hekmat Samir, MD

Patient ID: 97030	Patient Name English: Abeer Fekry Hussein Mohamed
٤٢ سنة Age:	2026/07/08 11:19:26 AM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A- 46- years old female patient with pathologically proven invasive ductal carcinoma grade II (-ve ER, -ve PR, +ve HER2 and Ki67= 20 %)

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild nodular background enhancement.

ACR: C (Heterogeneously dense breast parenchyma)

LEFT breast:

- **Low energy (Non contrast) images show**: Mid upper irregular shaped spiculated dense lesion associated with architectural distortion, underlying microcalcifications and diffuse skin thickening.
- **Recombined (Post contrast) images show**:
  - Corresponding large 11-12 o'clock irregular spiculated heterogeneously enhancing mass lesion with marked conspicuity, measuring about 4.6 x 6.5 cm along the maximum dimensions, associated with linear non-mass enhancement denoting intraductal extension reaching the nipple.
  - Multiple smaller heterogeneously enhancing masses of moderate conspicuity are scattered through the UOQ, retroareolar region and surrounding the index lesion along with intervening non-mass enhancement. The overall area of the enhancement (including the main index lesion and the surrounding lesions) measures about 12 cm along its maximum dimension (extending from 11:00-3:00).

RIGHT breast:

- **Low energy (Non contrast) images** No asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images**: no pathological mass or non-mass enhancement.
- Normal skin / nipple / areolar complex.

IMPRESSION:

- **LEFT BREAST:** Upper outer quadrant and retroareolar (from 11:00-3:00) multiple pathologically enhancing mass lesions associated with intraductal extension reaching the nipple, and diffuse edematous changes, as above described (BIRDAS VI)
- **RIGHT BREAST**: No pathological mass or non-mass enhancement (BIRADS I).

Best regards,

Dr. Sherihan fakhry , MD

Dr. Hekmat Samir, MD

Patient's Name:	**Abeer Fikri Hussein**
Date:	**2026-06-22**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- **New case of left breast cancer for metastatic work up.**

Findings:

- Left breast central mass is seen measuring 4x3.5cm.
- Increased skin thickness of left breast.
- Left axillary enlarged LNs largest 20mm.
- Clear right breast and right axilla.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Left thyroid lobe small nodule measuring 1cm.

OPINION:

- Left breast central mass.
- Increased skin thickness of left breast.
- Left axillary enlarged LNs.
- Clear right breast and right axilla.
- No lung metastasis.
- Left thyroid lobe small nodule.

جوستاف روسي

Patient's Name:	**Abeer Fikri Hussein**
Date:	**2026-06-22**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of enlarged size 20cm with fatty homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- D4 and D12 vertebral body lytic lesions.
- Right acetabular sclerotic lesion.

Opinion:

- **Fatty hepatomegaly with no hepatic metastasis.**
- **D4 and D12 vertebral body lytic lesions.**
- **Right acetabular sclerotic lesion.**
- **For bone scan.**

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Nuclear Medicine Unit وحدة الطب النووى

Abeer Fekry Hussien **Date:** ١٠/٠٨/٢٠٢٦

Tc ٩٩m MDP SKELETAL SCINTIGRAPHY

History

- A ٤٦-year-old lady with history of breast cancer.

Technique

- Anterior and posterior whole-body scans were acquired ٢ hours post injection of ٢٠ mCi of the tracer.

Findings

- No definite evidence of bone secondaries could be seen.
- Features suggestive of degenerative changes of the spine and big joints.

IMPRESSION

- The skeleton is scintigraphically free from metastases.

Appreciated,

Dr.Hossam Sekkina, M.Sc.

Dr. Alaa Nagi, MD
Nuclear Medicine